



LightCOM

Lighthouse Consortium on
Obesity Management

Study name: **Lighthouse Consortium on Obesity Management**
Short name: **LightCOM**

SOP title: **Standard Operating Procedure for Stool Sample
LightBAR**

SOP short title: **SOP on Stool Sample**

SOP Version: **1** Date: **09.04.2024**

Authored by: **Sam West**

Date:

TABEL OF CONTENTS

1. PURPOSE AND SCOPE.....	3
1.1 RECEIVING FAECAL SAMPLES.....	3
2. RECORDING SAMPLES RECEIVED	5
3. HISTORICAL RECORD OF SOP	6

1. PURPOSE AND SCOPE

This standard operating procedure (SOP) defines the process for receiving fixed stool samples. It also defines the processes for storing and recording samples once received.

It is the responsibility of the named person for each collection to ensure that samples are correctly stored and recorded.

This SOP applies to all personnel working with stool samples in the LightBAR trial.

1.1 RECEIVING FAECAL SAMPLES

UK:

At the CIDs, participants will be given a kit to complete at home. The sample kit will be explained to them by the person consenting the patient. The patient will provide a faecal sample in a tube containing 100% ethanol, wrap it in absorbent paper, seal it in a biobag and send it in a UN3373 labelled bio-sample cardboard box to the Kennedy Institute by Royal Mail.

Each project, and sometimes sub-project, is given a different identifier code (e.g. CMS034), and each tube is given an anonymous unique identifier code (LightBAR patient ID, visit and date) Each box should also contain the same identifier.

The unique identifier appearing on each tube will be provided by the LightBAR trial team. It is the responsibility of the recruitment site to keep a record matching the OCMS-generated identifier to the study patient ID. Please ensure that the participant is handed a kit with the correct identifiers on and enter the unique identifier code to the OpenClinica database before the kit is handed out. Thus, all potential initial samples are recorded before they are received.

The patient is instructed to collect stool sample into each sample tube containing 5 mL 100% ethanol.

Fixed faecal samples received by the OCMS will be fixed in 100% ethanol.

Upon arrival, the box is checked to ensure that the tamper proof seal is intact, and that there is no identifying information on the box.

If patients have added a name and/or address to the box this is immediately rendered invisible using a thick black marker pen.

Wearing gloves, the box is then opened and the contents inspected. The tube is carefully removed from the bag and wadding, and the tube number is double checked against the number listed on the box.

If the numbers do not match a note is made on the box of the actual tube number.

If the sample has not been returned correctly (e.g. the sample is not in the tube / the liquid has been removed from the tube) a decision will be made as to whether to store the sample or not.

If liquid has been removed from the tube then 100% ethanol will be added to the tube and this will be recorded.

If the sample is not returned in the tube this will be discarded immediately.

Samples are handled according to risk assessment KIR_RA-HUM_002 Work with human faeces.

The sample is stored in -80 freezer.

The time that the sample was frozen, and the date it was received are then written onto the box lid at the time of freezing and OpenClinica.

The plastic bag and wadding (and gloves if also returned) are discarded.

All boxes are then given to the OCMS Laboratory Technicians for recording, or, if not available, the OCMS Project Manager or an OCMS Experimental Lead.

DK:

Following consent, the patients are instructed to collect three stool samples before their clinical investigation days at CID1 and CID4. Patients are provided with a sample kit and a detailed written instruction for collection of the sample. The kit consists of three tubes designed for the purpose. Two of the tubes contain 5 mL of 96% ethanol and the third tube is empty.

After collection the sample tubes will be placed in transport tubes and packed with a gel-pack before storage in the patient's own freezer. The patient will bring the frozen package at their next visit, where clinical staff will receive it. The staff will ask the participant "Have you used any antibiotics or probiotics within the last 3 months?" and record the answer in OpenClinica.

For detailed instructions about the collection see

"12b.Patientinfo_Stool_Danish_LightBAR"

Upon receipt of the samples, they will be labelled and each sample tube will be marked with patient ID, date of sampling, study name, sample type, and CID-number (See 12c. *Håndteringskema_Bio_Materiale_DK_LightBAR* and 12d.

Labels_DK_LightBAR).

If patients have added a name or other identifiers to the tubes, it will be rendered invisible using a thick black marker pen.

The date for the sample collection will be entered in the OpenClinica Database.

Staff will wear gloves when the package is opened, and the tubes will be carefully removed from the bag and transport tube before the contents are inspected.

If the sample has not been returned correctly (e.g. the sample is not in the tube / the liquid has been removed from the tube) a decision will be made as to whether to store the sample or not:

- If liquid has been removed from the tube, then 5 ml of 96% ethanol will be added to the tube, and this will be recorded in the OpenClinica Database.
- If the sample is not returned in the tube it will be discarded immediately.

The samples are stored in -80°C freezer.

The date of the sampling is entered in the OpenClinica Database.

The plastic bags, transport tubes and the gel-pack etc. are discarded.

2. RECORDING SAMPLES RECEIVED

All samples are recorded onto the OpenClinica database, ideally on the same day as sample was received.

Access to each individual database is given by the LightBAR Trial Manager.

UK only:

As sample numbers are recorded in OpenClinica at the time of kit assembly data is added to the correct sample ID. Information stored will include the time and date of kit receipt, storage location of sample and type of sample.

Boxes are given to the OCMS Technicians preferentially, and if absent the OCMS Scientific Project Manager or the OCMS Experimental Lead, who are then responsible for the data input.

For OCMS samples where the study coordinator is not the OCMS Scientific Project Manager, the study coordinator may be informed by email whenever a sample has arrived (if relevant). The frequency of communication regarding kit arrival will be agreed with each study coordinator individually.

In no cases is any patient identifiable information received by the OCMS.

3. HISTORICAL RECORD OF SOP

Rev# / Date	Written/Revised By	Revision Description	Approval
0. Written	Sam West		
1. Edited	Sabrina Wielsøe	Edited to generic version	
2.			
3.			
4.			
5.			
6.			
7.			
8.			
9.			
10.			
Removed / replaced:			